

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

June 27, 2022

MEMORANDUM FOR:

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Assistant Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

THROUGH:

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Deputy Assistant Director of Clinical Services/Medical Director
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CDR (b)(6),(b)(7)(C),(b)(7)(F), PA-C
Chief, Investigations Unit
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FROM:

CDR (b)(6),(b)(7)(C),(b)(7)(F) RN
Senior Investigator
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

SUBJECT:

Mortality Review – Report of Findings
Roylan HERNANDEZ Diaz, (b)(6),(b)(7)(C)

Executive Summary:

On October 15, 2019, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of ICE detainee Roylan HERNANDEZ Diaz, (b)(6),(b)(7)(C). (b)(6),(b)(7)(C) During his detention period, ICE housed Mr. HERNANDEZ, a 43-year-old Cuban male, at Richwood Correctional Center (RCC) in Monroe, Louisiana (LA), from June 13 to October 15, 2019, the date of his death. An autopsy ruled Mr. HERNANDEZ' death was caused by hanging with ligature, consisting of a sheet wrapped tightly around the neck, and subsequent petechial hemorrhages of the epiglottic and laryngeal mucosa.

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On May 19, 2019, prior to Mr. HERNANDEZ' ICE custody and detention, U.S. Customs and Border Patrol (CBP) encountered Mr. HERNANDEZ as he applied for admission into the U.S. at the El Paso, Texas (TX) Paso Del Norte Port of Entry. CBP served Mr. HERNANDEZ with a Notice and Order of Expedited Removal, Form I-860, for an inadmissibility charge. During the initial interview, Mr. HERNANDEZ (b)(6),(b)(7)(C). On May 20, 2019, CBP transferred Mr. HERNANDEZ into ICE Enforcement and Removal Operations (ERO) custody in El Paso, TX. From May 21 to 23, 2019, ERO housed Mr. HERNANDEZ at the West Texas Detention Facility in Sierra Blanca, TX. On May 23, 2019, ERO transferred Mr. HERNANDEZ to the Tallahatchie County Correctional Facility in Tutwiler, Mississippi. On June 13, 2019, ERO transferred and housed Mr. HERNANDEZ to RCC, where he remained for the duration of his ICE custody.

On June 13, 2019, during his intake screening, Mr. HERNANDEZ denied any past or current mental health problems.

On October 10, 2019, Mr. HERNANDEZ declared hunger strike, and RCC housed him in administrative segregation for monitoring.

On October 15, 2019, at approximately 2:05 p.m., custody staff discovered Mr. HERNANDEZ hanging in his cell and unresponsive. Custody staff did not cut the ligature or initiate cardiopulmonary resuscitation (CPR). At approximately 2:14 p.m., a custody officer (CO) announced a medical emergency over the intercom system, and notified emergency medical services (EMS). Medical staff found Mr. HERNANDEZ hanging, unresponsive and pulseless. The Warden would not permit medical staff to cut the ligature or initiate CPR. At approximately 2:20 p.m., EMS arrived, cut Mr. HERNANDEZ' ligature, performed vital signs (VS) checks and cardiac monitoring, but Mr. HERNANDEZ did not have electrical activity in his heart, and his VS were unobtainable. EMS did not initiate CPR. At an unknown time, a deputy coroner arrived at RCC and pronounced Mr. HERNANDEZ deceased at 2:15 p.m.

A psychological autopsy is pending.

Mortality Findings:

As a result of Mr. HERNANDEZ' death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures, or practices are warranted; and identify issues that require further study.

A mortality review committee (MRC) determined that Mr. HERNANDEZ' medical care at RCC was not provided within the safe limits of practice. Specifically, the MRC determined RCC staff did not provide Mr. HERNANDEZ with mental health evaluations, did not provide emergency

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medical intervention, delayed access to medical services, and did not provide policies and procedures to the medical department.

The MRC convened on June 21, 2022, and consisted of the following IHSC members: Commander (CDR) (b)(6),(b)(7)(C),(b)(7)(F) Nurse Manager; Dr. (b)(6),(b)(7)(C),(b)(7)(F) Clinical Director; and Lieutenant Commander (LCDR) (b)(6),(b)(7)(C),(b)(7)(F) Nurse Practitioner.

The following summary of events was based on: 1) Mr. HERNANDEZ' RCC medical records and emergency medical services (EMS) report; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. HERNANDEZ' RCC detention file; 5) an on-site review and staff interviews conducted by factfinder, CDR (b)(6),(b)(7)(C),(b)(7)(F) at RCC on October 24 to 25, 2019; and 6) applicable ICE Detention Standards.

Detailed Medical Review

On June 13, 2019, at 11:05 a.m., a registered nurse (RN) documented Mr. HERNANDEZ' normal VS, prior history of tuberculosis clearance, and cleared him for general population (GP).

- At 2:05 p.m., an advanced practice provider (APP) completed Mr. HERNANDEZ' mental health screening with use of “Spanish” language services, and documented normal findings, except for a history of physical abuse in Cuba. The APP offered a referral to mental health, but Mr. HERNANDEZ refused.
- At 2:15 p.m., a physician (MD) completed Mr. HERNANDEZ' intake screening with the use of “Spanish” language services, and reviewed his accompanying TCCF transfer summary, which listed the following diagnoses: irritable bowel syndrome [(IBS); an intestinal disorder causing abdominal pain, gas, diarrhea, and constipation]; current mumps exposure with cohort status until June 26, 2019; and the following medications: dicyclomine (used for stomach and intestinal spasms) ten milligrams (mg), by mouth (PO), two times daily (BID), and senna (laxative) 8.6 mg, two tablets, PO, at bedtime (HS). The MD completed Mr. HERNANDEZ' physical exam, noted his report of chronic abdominal pain that he rated at two out of ten [(2/10); pain scale 0 to 10], and a history of physical abuse in Cuba. The MD documented Mr. HERNANDEZ' exam as “normal,” except for left (L) lower abdominal tenderness and diagnosed him with IBS. The MD continued Mr. HERNANDEZ' senna, increased the dicyclomine to 20 mg, PO, BID, scheduled him for a three-month follow-up (F/U), and cleared him for GP.
- An RN completed Mr. HERNANDEZ' prison rape elimination act (PREA) screening and documented negative findings.

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On June 18, 2019, Mr. HERNANDEZ submitted a sick call request for abdominal pain, which was unrelieved with his current medications.

On June 19, 2019, an RN attempted to evaluate Mr. HERNANDEZ for his abdominal pain, but he refused the evaluation, signing the refusal form, and eating.

- A CO completed an unusual occurrence report, noting Mr. HERNANDEZ missed six consecutive meals and refused his medications for two days, because “ICE failed to respond to his requests.”

On June 20, 2019, an RN completed Mr. HERNANDEZ’ health assessment, documented normal findings, except for his history of physical assault in Cuba (1994 and 2000), but did not refer Mr. HERNANEZ to the mental health provider.

- An RN completed Mr. HERNANDEZ’ pre-segregation health evaluation, prior to his placement in administrative segregation for hunger strike monitoring, and documented the following observations: disheveled appearance, disorganized and illogical thought processes, and a listless emotional state. The RN did not document Mr. HERNANDEZ’ VS, weight, initiation of hunger strike monitoring, and did not indicate whether Mr. HERNANDEZ was cleared for segregation placement.

On June 21, 2019, an RN evaluated Mr. HERNANDEZ, documented his blood pressure (BP) - 138/98 millimeters of mercury [(mmHg); normal range is 90/60 to 120/80 mmHg], and his pulse (P) - 98 beats per minute [(bpm); normal 60-100 bpm]. The RN observed Mr. HERNANDEZ drinking two cups of juice and gave him a bottle of Ensure (nutritional supplement).

- A licensed practical nurse (LPN) evaluated Mr. HERNANDEZ during segregation rounds, and his VS were: BP - 124/86 mmHg, P - 90 bpm, and weight - 160.6 pounds (lbs.). The LPN collected a urine specimen for urinary analysis, documented Mr. HERNANDEZ’ intake of an eight-ounce bottle of Ensure, and consumption of 25 - 30 percent (%) of his lunch.
- An MD evaluated Mr. HERNANDEZ during segregation rounds, documented normal exam findings, except for L lower abdominal tenderness, diagnosed Mr. HERNANDEZ with IBS, depression (not suicidal), ordered him a bland diet with extra vegetables, and discharged him to GP.
- An LPN notified an APP, who approved Mr. HERNANDEZ to remain in the segregation unit over the weekend for continued food and fluid intake monitoring. The LPN did not chart the verbal order in the medical record.

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On June 23, 2019, an LPN documented Mr. HERNANDEZ refused his medications (dicyclomine and senna) and refused to sign a refusal form.

On June 24, 2019, an MD evaluated Mr. HERNANDEZ for his complaint of receiving spicy, low fiber foods. The MD documented normal exam findings, except for a BP - 160/85 mmHg. The MD ordered Mr. HERNANDEZ a bland diet, psyllium fiber powder laxative, and to recheck his BP in thirty minutes. No one rechecked Mr. HERNANDEZ' BP.

On June 30, 2019, an LPN administered Mr. HERNANDEZ a measles, mumps, and rubella vaccination.

On July 5, 2019, Mr. HERNANDEZ submitted a sick call request for an MD/APP evaluation because his dicyclomine worsened his symptoms. An RN submitted a referral for Mr. HERNANDEZ to receive an MD/APP evaluation.

- An MD evaluated Mr. HERNANDEZ for his sick call request and documented the following findings: abdomen soft and non-tender to palpation. The MD diagnosed Mr. HERNANDEZ with “chronic worry about colon cancer and inflamed colon,” discontinued his dicyclomine, and ordered three samples for hemoccult testing (test for detecting blood in the stool), that showed normal results (dated August 8 and 9, 2019). Mr. HERNANDEZ continued to receive the dicyclomine until July 8, 2019. No one performed the hemoccult testing.

On July 13, 2019, an RN evaluated Mr. HERNANDEZ during sick call for an ingrown toenail and pain. The RN documented normal exam findings, normal VS, and released Mr. HERNANDEZ to his housing unit with instructions to return if his condition worsened.

On July 30, 2019, an RN evaluated Mr. HERNANDEZ during sick call for itching. The RN documented normal exam findings, normal VS, instructed Mr. HERNANDEZ to use body lotion after showering, and to return to the clinic as needed.

On August 6, 2019, an RN evaluated Mr. HERNANDEZ during sick call for persistent abdominal pain he rated 7/10 and his request to change dicyclomine to an alternative treatment. The RN documented normal VS and referred Mr. HERNANDEZ to the MD/APP for further evaluation.

On August 8, 2019, an APP evaluated Mr. HERNANDEZ. The APP documented normal findings, diagnosed him with abdominal pain, reordered hemoccult testing, and scheduled Mr. HERNANDEZ for a F/U appointment in two weeks. The APP did not order Mr. HERNANDEZ any additional treatment.

On August 26, 2019, an APP completed Mr. HERNANDEZ' scheduled F/U visit,

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documented normal exam findings, except for L lower abdominal tenderness, and diagnosed him with chronic IBS. The APP discontinued the senna, ordered pinaverium bromide (for intestinal spasms) one tablet, PO, BID, instructed Mr. HERNANDEZ to F/U as needed, and recommended a referral to the mental health provider. However, Mr. HERNANDEZ refused the mental health referral, and the APP provided education on stress relieving techniques. The APP did not obtain a signed refusal. No one transcribed the pinaverium bromide to the medication administration record.

On September 4, 2019, an RN triaged Mr. HERNANDEZ' sick call request for a mental health consult and submitted a referral to mental health.

On September 10, 2019, an LPN evaluated Mr. HERNANDEZ during sick call for persistent abdominal pain, since the discontinuation of the senna and not taking the pinaverium bromide. The LPN documented normal exam findings and VS, except for sluggish bowel sounds. As a result, the LPN referred him to the MD/APP for further evaluation.

On September 13, 2019, an MD evaluated Mr. HERNANDEZ for his complaint of persistent abdominal pain. The MD documented normal exam findings and VS, except for tenderness to the lower half of Mr. HERNANDEZ' abdomen. The MD diagnosed Mr. HERNANDEZ with chronic constipation, abdominal pain, discontinued the pinaverium bromide, and re-ordered senna 8.6 mg, one tablet, PO, every evening.

On September 14, 2019, an RN documented Mr. HERNANDEZ refused his mental health appointment, so the RN obtained a signed refusal.

On October 10, 2019, an RN completed Mr. HERNANDEZ' pre-segregation health evaluation for his threat to go on hunger strike. Mr. HERNANDEZ refused his VS and weight assessment. The RN documented a normal physical and mental health assessment, except for his withdrawn emotional state, and cleared Mr. HERNANDEZ for administrative segregation housing placement. The RN did not place a mental health referral.

On October 11, 2019, at 4:00 a.m., an RN noted Mr. HERNANDEZ resting and without any complaints.

- At 7:00 a.m., an RN documented Mr. HERNANDEZ refused his VS check, did not report any complaints, and in no distress.
- At 2:20 p.m., an unknown author documented Mr. HERNANDEZ did not eat lunch, did not report any complaints, and in no distress.

On October 12, 2019, at 10:50 a.m., an RN documented Mr. HERNANDEZ did not voice any complaints or requests.

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- At 12:30 p.m., an RN documented a CO confirmed Mr. HERNANDEZ ate dinner last night and his breakfast in the morning.
- At 8:30 p.m., an unknown author documented Mr. HERNANDEZ did not voice any complaints and in stable condition.

On October 13, 2019, at 9:05 a.m., an RN noted Mr. HERNANDEZ resting, not in any distress, responsive to verbal stimuli, but non-communicative with staff.

- At 11:05 p.m., an unknown author documented Mr. HERNANDEZ refused lunch and dinner on October 12, 2019, and all three meals on October 13, 2019. In addition, Mr. HERNANDEZ did not voice any complaints and rested in no acute distress, with even and unlabored respirations.

On October 14, 2019, at 8:00 a.m., an RN documented Mr. HERNANDEZ awake, alert, oriented, and in no distress. Mr. HERNANDEZ refused his VS assessment and refused to sign the hunger strike treatment consent form.

On October 15, 2019, at 12:00 p.m., an RN documented Mr. HERNANDEZ refused his breakfast, lunch, medical assessment, to shower, and blood work for laboratory testing. The RN notified ERO and IHSC that Mr. HERNANDEZ missed ten consecutive meals.

- At 1:55 p.m., a CO completed segregation rounds; however, the CO did not document whether Mr. HERNANDEZ was observed through the cell door window.
- At 2:04 p.m., a CO Captain walked by Mr. HERNANDEZ' cell, smelled an odor, looked through the cell door window but did not visualize Mr. HERNANDEZ.
- At 2:05 p.m., the CO Captain returned to Mr. HERNANDEZ' cell, opened the door, found Mr. HERNANDEZ hanging, unresponsive, with a sheet tied around his neck, and the other end of the sheet tied to the rail of the top bunk. The CO Captain requested his supervisor and the Warden to respond to Mr. HERNANDEZ' cell over radio transmission.
- At 2:14 p.m., the Warden instructed the CO Captain to call for an ambulance, and announce a medical emergency. Medical staff responded, found Mr. HERNANDEZ hanging with a sheet around his neck, unresponsive, pulseless, with a pale face, and mottled skin on his arms. An LPN retrieved a cut-down tool to cut the sheet and lower Mr. HERNANDEZ to the ground to start CPR; however, the Warden denied medical staff access to Mr. HERNANDEZ and stated, "based on Mr. HERNANDEZ' appearance and condition, he is dead, and the cell was now a crime scene." The medical staff made

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repeated requests to cut down Mr. HERNANDEZ and initiate CPR, but the Warden denied every request.

- At 2:20 p.m., EMS arrived, cut down Mr. HERNANDEZ, performed VS checks, and cardiac monitoring, but Mr. HERNANDEZ did not have electrical activity in his heart, his VS were unobtainable, and EMS did not initiate CPR.
- At 2:30 p.m., RCC notified the Ouachita Parish Sheriff's Department.
- At 3:09 p.m., the sheriff's deputies arrived, completed their investigation, and notified the coroner's office. A deputy coroner arrived and pronounced Mr. HERNANDEZ deceased at an 2:15 p.m.

Detention Standards used for this review: National Detention Standards 2000 and the National Commission on Correctional Health Care (NCCHC) Standards, 2018. The following are identified program strengths, weaknesses, and recommendations found during this review.

Strengths

Strengths identified during this investigation were: an APP or MD evaluates detainees with acute or chronic health problems on the day of arrival to RCC; and medical staff responded to Mr. HERNANDEZ' cell within four minutes of the medical emergency announcement.

Weaknesses

1. Emergency Response

- On October 15, 2019, at 2:05 p.m., a CO Captain discovered Mr. HERNANDEZ hanging, did not initiate CPR, and did not announce an emergency until 2:14 p.m.
- The Warden denied medical staff access to Mr. HERNANDEZ.

Applicable standards of care for this finding:

- NDS 2000, Medical Care, III, D. *Medical Screening*.
- NCCHC 2018, J-D-07, *Emergency Services and Response Plan*.

2. Infection Control

- On June 13, 2019, an RN, an APP, and an MD reviewed Mr. HERNANDEZ' transfer summary indicating continuation of his mumps cohort, but did not follow this order for continuity of care purposes.

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Applicable standards of care for this finding:

- NCCHC 2018, J-B-02, *Infectious Disease Prevention and Control*.

3. Policies and Procedures

- LaSalle Corrections Corporation assumed operations of RCC in April 2019, but did not provide staff with policies and procedures until October 2019.

Applicable standards of care for this finding:

- NDS 2000, Medical Care, I. *Policy*.
- NCCHC 2018, J-A-05, *Policies and Procedures*.

4. Special Management Unit (Administrative Segregation)

- On October 15, 2019, the segregation CO did not observe or make direct contact with Mr. HERNANDEZ for three hours and thirty-nine minutes. During this time frame, the CO Captain discovered Mr. HERNANDEZ hanging.

Applicable standards of care for this finding:

- NDS 2000, Special Management Unit (Administrative Segregation), D. *Conditions of Administrative Segregation (Basic Living Standards) Item 12*.

5. Interpreter Services

- RCC staff did not consistently use language services when communicating with Mr. HERNANDEZ, who spoke only Spanish.

Applicable standards of care for this finding:

- NDS 2000, *Medical Care*.
- NCCHC 2018, J-E-01, *Information on Health Services*.

6. Medical Records

- The nurses did not consistently document Mr. HERNANDEZ' sick call evaluation findings in his medical record, when addressing his sick call complaint.
- On June 13, 2019, an MD documented medication orders in Mr. HERNANDEZ' medical record, but nursing staff did not note and transcribe the orders until June 15, 2019.

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- On June 20, 2019, an RN completed Mr. HERNANDEZ' administrative segregation clearance, but did not document the encounter in his medical record.
- On June 21, 2019, an RN did not time a nursing note entry and left the entry incomplete.
- On June 21, 2019, an LPN received a verbal order from an APP, but did not document the order in Mr. HERNANDEZ' medical record.
- On June 24, 2019, an MD documented ordered a recheck of Mr. HERNANDEZ' blood pressure, but nursing staff did not perform the order.
- On July 5, 2019, an MD documented hemocult test orders in Mr. HERNANDEZ' medical record, but nursing staff did not perform the orders.
- On August 26, 2019, an APP documented medication orders in Mr. HERNANDEZ' medical record, but nursing staff did not note or transcribe the orders.

Applicable standards of care for this finding:

- NDS 2000, Medical Care, I. Policy.
- NCCHC 2018, J-A-08, *Health Records*.

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director of Health Care Compliance (DAD-HCC).
- The IHSC DAD-HCC will share these findings through appropriate communication channels to ICE, RCC's administrator, and health authority for review and any action(s) deemed appropriate.
- The respective IHSC HCC Unit and ICE will ensure the corrective action plan, if applicable, is implemented and sustained.

End of report.